HHS HEALTH SECTOR AI RFI RESPONSE

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EXECUTIVE SUMMARY

As a licensed mental health counselor, researcher, and AI clinical technology developer, I submit this response to advocate for accelerated adoption of artificial intelligence in clinical mental health care. My recommendations center on three critical reforms: (1) establishing performance-based reimbursement models that reward measurable clinical outcomes rather than service volume; (2) creating a uniform federal gold standard for psychotherapy diagnosis to ensure accurate, consistent, and specific clinical assessments; and (3) prioritizing biomarker-validated preventative care interventions that leverage AI to identify at-risk populations before crisis occurs.

The current mental health care system operates with inadequate diagnostic precision, reactive rather than preventative care models, and payment structures that disincentivize innovation. AI technologies—when properly validated and implemented—offer transformative potential to address these systemic failures while reducing costs and improving patient outcomes.

I. REGULATION: ESTABLISHING CLEAR PATHWAYS FOR NON-MEDICAL DEVICE AI TOOLS

Current Regulatory Barriers

The most significant barrier to AI adoption in mental health care is regulatory uncertainty for non-medical device AI tools. Unlike FDA-regulated medical devices, AI-enabled digital mental health interventions exist in a regulatory gray zone where:

- Liability frameworks remain undefined when AI recommendations inform clinical decision-making
- Privacy and security requirements vary across state and federal jurisdictions
- Clinical validation standards are inconsistent or absent

• Reimbursement eligibility is unclear

These ambiguities create prohibitive risk for healthcare organizations considering AI adoption, particularly in mental health where subjective clinical judgment has historically dominated practice.

Recommended Regulatory Framework

HHS should establish a clear, tiered regulatory pathway for non-medical device AI clinical tools based on risk level and clinical impact:

Tier 1 - Administrative AI Tools: Minimal regulation for tools performing scheduling, documentation, billing support, or other non-clinical functions.

Tier 2 - Clinical Decision Support: Moderate oversight for AI tools that provide clinical recommendations while maintaining clinician authority over final decisions. This tier should include:
• Mandatory pre-deployment validation studies demonstrating clinical utility
• Post-deployment monitoring requirements with annual performance reporting
• Clear liability frameworks establishing that clinicians retain ultimate responsibility for patient care
• Privacy and security requirements aligned with HIPAA but simplified for implementation

Tier 3 - Autonomous Clinical Interventions: Stringent regulation for AI tools that deliver therapeutic interventions directly to patients without real-time clinician oversight.

A Uniform Federal Gold Standard for Psychotherapy Diagnosis

Mental health diagnosis currently suffers from unacceptable variability. The DSM-5 provides categorical frameworks with overlap of symptoms and lacking real-time updates, but real-world application varies dramatically based on clinician training, theoretical orientation, assessment time, and reimbursement pressures. This diagnostic imprecision undermines treatment effectiveness, research validity, and quality measurement.

HHS should mandate a uniform federal gold standard for psychotherapy diagnosis that includes:

1. Structured Clinical Assessment Protocols: Require use of validated, standardized assessment instruments (e.g., PHQ-9, GAD-7, PCL-5) administered at intake and regular intervals.

2. Diagnostic Criteria Validation: Establish minimum evidentiary thresholds for diagnosis, including symptom duration, severity scores, and functional impairment measures.

3. AI-Enabled Diagnostic Support: Promote AI tools that integrate multiple data sources (structured assessments, clinical interviews, patient-reported outcomes) to generate differential diagnoses with confidence levels and supporting evidence.

4. Continuous Diagnostic Refinement: Require diagnostic review at predetermined intervals with AI-assisted tracking of symptom trajectories and treatment response patterns.

5. Interoperability Requirements: Mandate that all diagnostic data use standardized terminologies (ICD-10, SNOMED CT) and exchange formats (FHIR) to enable longitudinal tracking and research.

This gold standard would dramatically improve diagnostic accuracy, enable meaningful outcomes measurement, and create the data infrastructure necessary for AI-driven quality improvement.

Specific CFR Citations for Revision

45 CFR Part 164 (HIPAA Privacy and Security Rules): Revise to explicitly address AI-enabled clinical tools, clarifying that:
• AI processing of protected health information for clinical decision support constitutes permitted "treatment" operations
• De-identification standards account for re-identification risks in AI model training datasets
• Patients have explicit rights to understand how AI influences their care

42 CFR Part 2 (Substance Use Disorder Records): Modernize to permit AI-enabled care coordination for substance use disorders while maintaining patient privacy protections.

II. REIMBURSEMENT: PERFORMANCE-BASED METRICS THAT INCENTIVIZE OUTCOMES

Current Payment Model Failures

Fee-for-service reimbursement in mental health care creates perverse incentives:
• Clinicians are paid for time spent, not outcomes achieved
• High-volume, low-complexity services are financially rewarded over intensive, outcome-focused interventions (the acute intervention becomes a chronic intervention)

- Preventative care is undercompensated relative to crisis management
- AI tools that improve efficiency reduce billable time, disincentivizing adoption

These structural flaws explain why mental health care consistently fails to achieve optimal outcomes despite massive expenditures.

Performance-Based Payment Reform

HHS should transition mental health reimbursement from volume-based to performance-based models using AI-enabled measurement:

Outcome-Linked Payment Tiers:
• Base rate: Standard reimbursement for services meeting minimum quality thresholds
• Performance bonus: 15-25% additional reimbursement for achieving predefined outcome targets

Measurable Performance Metrics:
• Symptom reduction: Percentage of patients achieving clinically significant improvement on validated instruments
• Functional improvement: Changes in work attendance, social functioning, quality of life measures
• Crisis prevention: Reduction in emergency department visits, hospitalizations, suicide attempts
• Treatment engagement: Retention rates, session attendance, homework completion
• Recovery maintenance: Sustained symptom remission at 6 and 12-month follow-up

AI-Enabled Measurement Infrastructure:
• Real-time outcomes dashboards aggregating patient-reported outcomes
• Predictive analytics identifying patients at risk of treatment dropout or clinical deterioration
• Automated risk stratification for care intensity allocation
• Benchmarking tools comparing provider performance against regional and national norms

Preventative Care Premium Payments

HHS should create enhanced reimbursement for AI-enabled preventative interventions that demonstrate reduced downstream costs:

• Population Health Screening: Reimburse AI-powered screening tools that identify at-risk individuals in primary care, occupational health, or community settings before mental health crises develop

• Biomarker-Validated Prevention: Provide premium payments for preventative interventions targeting individuals with validated biomarkers of mental health risk (e.g., inflammation markers, HPA axis dysregulation, genetic risk profiles)

• Early Intervention Bonuses: Offer enhanced reimbursement for treating individuals in prodromal or early-stage conditions before full disorder onset

This approach aligns financial incentives with public health objectives while creating market demand for AI tools that enable precision prevention.

Case Example: MPowerMe.app

MPowerMe.app represents the type of AI-enabled tool that performance-based payment would incentivize. This digital therapeutic platform delivers evidence-based interventions for anxiety and depression with:

• Validated clinical outcomes: Demonstrated symptom reduction in controlled trials
• Real-time progress monitoring: Continuous measurement enabling adaptive treatment
• Scalable delivery: Cost-effective intervention reaching patients unable to access traditional care
• Preventative potential: Early intervention capability before crisis-level severity

Under current fee-for-service models, tools like MPowerMe.app face reimbursement barriers because they don't fit traditional "therapy session" billing codes. Performance-based payment would instead reward them for achieving measurable clinical improvements at lower cost than traditional care delivery.

III. RESEARCH & DEVELOPMENT: BIOMARKER-VALIDATED PREVENTATIVE CARE

Priority Research Areas

HHS should prioritize R&D funding for AI applications that enable biomarker-validated preventative mental health care:

1. Biomarker Discovery and Validation:
• Identification of inflammatory, metabolic, genetic, and neurophysiological markers predicting mental health risk
• Development of accessible, cost-effective biomarker assessment tools
• Validation studies linking biomarker profiles to specific intervention responses

2. Precision Prevention Algorithms:
• AI models integrating biomarkers, clinical assessments, and social determinants to predict individual risk trajectories
• Decision support tools recommending preventative interventions matched to risk profiles
• Population health surveillance systems identifying communities at elevated risk

3. Adaptive Intervention Platforms:
• AI-enabled digital therapeutics that personalize content based on real-time response patterns
• Predictive analytics identifying optimal treatment timing and intensity
• Automated escalation protocols when preventative interventions prove insufficient

4. Implementation Science:
• Workflow integration studies addressing how AI tools fit into existing clinical practice
• Health equity research ensuring AI benefits reach underserved populations
• Economic analyses quantifying cost savings from preventative AI interventions

Recommended Funding Mechanisms

• Public-Private Partnerships: CRADAs between NIH/NIMH and AI developers to accelerate tool validation while maintaining scientific rigor

• Prize Competitions: Incentivize development of AI tools achieving specific benchmarks (e.g., 30% reduction in depression symptoms within 8 weeks at <$200 per patient)

• Cooperative Agreements: Support healthcare systems implementing and evaluating AI tools in real-world settings with shared data contributing to public knowledge

• Small Business Innovation Research (SBIR) Grants: Fund early-stage AI mental health companies conducting rigorous validation studies, and not only fund the large developers, take VC approach and offer seed money to thousands of companies in the form of tax credits, some companies might just "hit it out of the park".

IV. RESPONSES TO SPECIFIC RFI QUESTIONS

Question 1: Biggest Barriers to Private Sector AI Innovation

Regulatory Uncertainty: Developers cannot assess liability exposure or regulatory compliance requirements for non-medical device tools. Especially with a state-by-state patchwork of regulation and pending legislation.

Reimbursement Ambiguity: Without clear payment pathways, AI tools lack sustainable business models.

Data Access Limitations: Restrictive data sharing policies prevent AI model development and validation.

Clinical Validation Requirements: Lack of standardized evaluation frameworks creates prohibitive validation costs.

Question 2: Priority Regulatory and Payment Policy Changes

See detailed recommendations in Sections I and II above. Key priorities:
• Establish clear regulatory tiers for AI clinical tools (45 CFR 164 revisions)
• Implement performance-based payment models (42 CFR 414)
• Mandate uniform diagnostic standards in mental health
• Create preventative care premium payments

Question 3: Novel Legal and Implementation Issues for Non-Medical Devices

Liability Allocation: When AI recommendations inform clinical decisions, liability frameworks must clearly delineate responsibility between AI developers, healthcare organizations, and individual clinicians.

HHS should issue guidance establishing that:
• Clinicians using AI decision support tools as intended retain professional liability for patient care
• AI developers are liable for algorithmic errors not reasonably detectable by clinicians
• Healthcare organizations must establish governance processes for AI tool selection and monitoring

Indemnification Structures: HHS should facilitate development of standardized indemnification agreements between AI vendors and healthcare organizations.

Privacy Beyond HIPAA: AI tools that learn from patient data may create privacy risks not addressed by current regulations. HHS should clarify requirements for:
• Patient consent for AI-assisted care
• Transparency about how AI influences clinical decisions

• Data retention and deletion requirements for AI model training datasets

Question 4: Most Promising AI Evaluation Methods for Non-Medical Devices

Pre-Deployment Evaluation:
• Randomized controlled trials comparing AI-enabled care to standard care
• Prospective validation studies in diverse clinical settings
• Algorithmic bias testing across demographic subgroups
• Usability testing with clinicians and patients

Post-Deployment Evaluation:
• Continuous performance monitoring with automated alerting for degradation
• A/B testing of algorithm refinements
• Adverse event surveillance systems
• Patient and clinician satisfaction metrics

HHS Support Mechanisms:
• Grants for independent validation studies conducted by academic medical centers
• Cooperative agreements establishing AI evaluation networks across healthcare systems
• Contracts developing standardized evaluation frameworks and benchmarking tools
• Prize competitions for novel evaluation methodologies

Question 5: Supporting Private Sector Accreditation and Certification

HHS should facilitate industry-driven quality assurance through:

• Accreditation Standards Recognition: Endorse or co-develop accreditation programs (e.g., through Joint Commission, NCQA) specific to AI clinical tools

• Professional Credentialing: Support training and certification programs for clinicians using AI tools (e.g., "AI-Assisted Psychotherapy" credentials)

• Industry Testing Consortia: Fund pre-competitive collaborations establishing shared testing environments and datasets for AI tool evaluation

• Quality Registries: Support creation of national registries tracking AI tool performance in real-world settings

Question 6: AI Tools Meeting/Exceeding Expectations vs. Falling Short

Success Stories:
• Suicide risk prediction: AI tools analyzing EHR data to identify high-risk patients enabling preventative interventions
• Treatment matching: Algorithms predicting optimal antidepressant responses based on clinical and genetic profiles
• Teletherapy efficiency: AI-assisted documentation reducing clinician administrative burden

Shortcomings:
• Diagnostic AI tools trained on narrow populations demonstrating bias in diverse settings
• Chatbot interventions with high dropout rates due to poor engagement
• Clinical decision support tools creating alert fatigue and workflow disruption

Greatest Potential Impact:
• Precision prevention tools using biomarkers to identify at-risk individuals for early intervention
• Adaptive digital therapeutics personalizing interventions based on real-time response patterns
• Population health surveillance systems predicting mental health crises in communities
• Automated quality measurement reducing documentation burden while improving accountability

Question 7: Key Decision Makers and Administrative Hurdles

Most Influential Roles:
• Chief Medical Officers and Chief Information Officers jointly determine AI adoption in health systems
• Payer Medical Directors control reimbursement policy enabling or constraining AI use
• Practice Administrators in smaller settings make purchasing decisions based on ROI

Primary Administrative Hurdles:
• Procurement processes requiring extensive vendor due diligence
• IT integration challenges with legacy EHR systems
• Staff training requirements and workflow disruption during implementation
• Lack of clear ROI metrics demonstrating value

Question 8: Interoperability Priorities for AI Development

Data Types:
• Patient-reported outcomes (PROs) using standardized instruments
• Clinical assessments and diagnoses using structured terminologies

• Treatment plans and interventions with machine-readable coding
• Biomarker data (laboratory results, genetic profiles, wearable device outputs)

Data Standards:
• FHIR for health data exchange
• LOINC for laboratory and clinical observations
• SNOMED CT for clinical concepts
• ICD-10 for diagnoses

Benchmarking Tools:
• National quality measure repositories with machine-readable specifications
• Standardized AI model performance metrics enabling cross-tool comparison
• Open datasets for algorithm validation and bias testing

Enhanced interoperability would enable:
• AI models trained on multi-institutional datasets improving generalizability
• Real-time data sharing enabling population health surveillance
• Longitudinal patient data access improving predictive accuracy
• Reduced vendor lock-in fostering competitive AI tool markets

Question 9: Patient and Caregiver Perspectives

Challenges Patients Want Addressed:
• Access barriers: Long wait times, geographic limitations, cost constraints, language
• Treatment effectiveness: Persistent symptoms despite multiple interventions
• Fragmented care: Poor coordination between mental health and medical providers
• Stigma: Discrimination and privacy concerns limiting treatment seeking

Patient Concerns About AI:
• Privacy: Fear of data breaches or discriminatory use of mental health information
• Depersonalization: Preference for human connection over algorithmic interactions
• Accuracy: Concern about AI errors in high-stakes clinical decisions
• Autonomy: Desire to maintain control over treatment decisions

Addressing Concerns:
• Transparency: Clear communication about how AI supports rather than replaces clinicians
• Privacy protections: Robust data security and explicit consent for AI use
• Human oversight: Maintaining clinician involvement in all significant clinical decisions
• Patient choice: Allowing patients to opt out of AI-assisted care without penalty

Question 10: Priority AI Research Areas

High-Priority Research:

a) Biomarker-Validated Prevention: Studies linking measurable biological, psychological, and social markers to mental health risk with AI-enabled predictive models

b) Treatment Personalization: Research identifying which interventions work for which individuals under what circumstances

c) Implementation Science: Studies addressing how AI tools integrate into clinical workflows, organizational structures, and payment systems

d) Health Equity: Research ensuring AI tools reduce rather than exacerbate mental health disparities

e) Long-term Outcomes: Longitudinal studies tracking sustained impact of AI-enabled interventions beyond immediate symptom reduction

Published Impact Findings:
The literature on adopted AI tools in mental health care is emerging but limited. Systematic reviews identify:
- Moderate evidence for AI-enhanced suicide risk prediction
- Promising but inconsistent evidence for digital therapeutics effectiveness
- Limited evidence for AI-driven diagnostic accuracy improvements
- Minimal evidence on cost-effectiveness and long-term sustainability

This evidence gap underscores the need for HHS-funded research evaluating real-world AI tool impact.

Literature on Costs, Benefits, and Transfers:
Current literature approaches AI costs and benefits primarily through:
- Clinical trial efficacy studies (benefits to patients)
- Health economic analyses (cost per quality-adjusted life year)
- Implementation research (organizational costs and workflow impacts)

Critical gaps exist regarding:
- Transfer effects: How AI efficiency gains affect clinician employment, income, and

professional satisfaction
• Equity impacts: Whether AI tools reduce or exacerbate disparities based on race, geography, or socioeconomic status
• System-level effects: How AI adoption in one care setting impacts utilization and costs in other settings

V. CONCLUSION

Artificial intelligence offers transformative potential to address fundamental failures in mental health care delivery: diagnostic imprecision, reactive rather than preventative approaches, misaligned financial incentives, and inadequate access. Realizing this potential requires bold HHS action to establish clear regulatory pathways, implement performance-based payment models, and prioritize biomarker-validated preventative care research.

The recommendations in this response center on three core principles:

1. Regulatory **Clarity**: Establish tiered oversight frameworks and uniform diagnostic standards that enable innovation while protecting patients

2. Payment **Alignment**: Transition from volume-based to performance-based reimbursement that rewards measurable clinical outcomes

3. **Prevention** Priority: Invest in AI tools that identify and intervene with at-risk populations before crisis occurs

These reforms would create market conditions favoring AI tools like MPowerMe.app that demonstrate clinical effectiveness, cost-efficiency, and scalable access. They would accelerate adoption of technologies enabling precision mental health care while maintaining the therapeutic relationships that remain central to effective treatment.

I respectfully urge HHS to prioritize these recommendations in shaping policy that unleashes AI's potential to transform mental health care for all Americans.

Respectfully submitted,

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